

Ref by Mr. Kishore

Dr Agarwals

Eye Hospital

Dr. Padma Pruthi

Operation Theatre Top Sheet

1. Name of the Patient : Mrs V. Semathi Age : 63 Sex : M (F)
e/o Mr. Senthosh Kumar
2. Mobile Number : 9884422002 / 9884422003 M.R. No. :
3. Eye : RE vit + MP + ER MP + cat + CMT-239329/23
AFL

CLINICAL DETAILS :

Vision :

Pupil : Dil / Mid Dil :

Fundus :

Syringing :

Specular Count : 2649

OP :

Grade of CAT :

Other Features :

Conj Swab if done : + / -

WTW (For Glued IOL case) :

Other Eye Significant Findings :

One Eyed : YES ☐ NO ☒

TYPE OF SURGERY : PDR + MP + ER MP
Feat + IOL

TYPE OF IOL : AFL

GVP : Advised / Not Advised

Date of Surgery : 16/9/23

Time of Surgery : 9.00 - 9.30

Power of IOL : Naspv + 21.5D

If Advised - Obtained / Not Obtained

MEDICAL & SURGICAL HISTORY : MI / CABG / STROKE / BR ASTHMA / DM / HTN

PHYSICAL FITNESS : Obtained (Date : 14/9/23) / Not Obtained

TYPE OF ANAESTHESIA : LA ☒ TA ☐ GA ☐

XYLOCAINE TEST DOSE : Given ☒ Not Given ☐

PAYMENT MODE : Insurance / Cash / Company (.....)

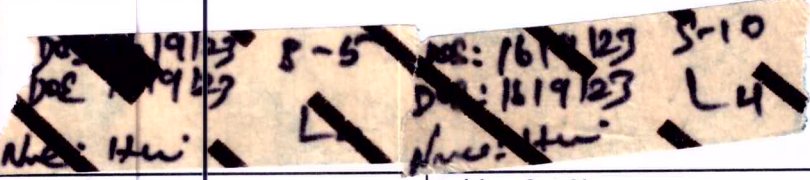
Amount : Total : 28000 + 50,000
Discount 3000
Paid :

Balance to be collected : 25,000 + 50,000

REMARKS :



 Dr. Agarwal's Eye Hospital NO:222, TTK ROAD, ALWARPET, CHENNAI-18.			
OPERATION NOTES			
Patient Name: <u>Mrs. V. Sumathi</u>		Ref NO: <u>CMH 239329</u>	
Date: <u>16/9/23</u>		Operating Eye: <u>RE</u>	
Anaesthesia Details			
Anaesthesia type	Topical Anaesthesia	Local Anaesthesia	General Anaesthesia
Anaesthesia given by/Time:	<u>DR. PRUSTON</u>		
Complications If any	<u>-</u>		
Intra operative Note			
Theatre No:	<u>6</u>	Case No:	
Surgeon:	<u>DR. PADMA/MALE</u>	Assistant:	<u>DR. J. CORRAO</u>
Starting Time	<u>12 PM</u>	Finishing Time	
Type of Surgery/Lens	<u>RE - Phaco + PCIO + PPV + ERMP + BUMP + Au.</u>		
Vitreous hemorrhage 1. Transconjunctival 23/25 gauge sclerotomy made at 3/3.5 mm from limbus at ITQ 2. Infusion cannula fixed at ITQ 3. Transconjunctival sclerotomies made at 3/3.5 mm from limbus at STQ and SNQ 4. Core Vitrectomy done 5. IVTA assisted PVD induction done and vitrectomy completed 6. Bleeding vessels/prolif cauterized 7. Prolif delaminated/dissected using scissors 8. Prolif trimmed using cutter 9. PRP done 10. FGE done 11. Silicon oil /gas injected. 12. Sclerotomies were self sealing 13. Sub conjunctival Genta+Deca given 14. Eye patched			
Device	CTR	Iris Hook	Capusular Hook
	NIL		
Suture	YES	NO	
	Material NO:		Suture count:
Complication If Any	<u>-</u>		
IOL Sticker	SN: AH23076706084 REF: nAs207 LOT: 23-07100 ØB: 6.00 mm POWER: 21.50 D M: 2023-08 ØT: 12.50 mm A.Constant: 118.00 D: 2027-07 CE 2460 nAspro AI OPTICS Pvt. Ltd., INDIA		
Surgeon Signature/seal: <u>DR. PADMA MALE</u>			

		Dr. Agarwal's Eye Hospital	
		NO.222, TTK ROAD, ALWARPET, CHENNAI-18.	
Patient Name:		Ref No:	Date:
Intra Cameral	Batch Number	Expiry Date	
Trypan Blue			
Keratome			
Side port			
Crescent			
Viscoat			
Viscon	DUR 462	5/25	
Contosol / Intasol	22F59	5/25	
Gentamycin	ANH 0279	11/25	
Dexamethasone	372-1142	10/24	
Epitrate B. blue	3E373	4/25	
Oculan			
Pilocarpine			
Sterilization Sticker			
Phaco Probe No. -	IA Probe No.-	Tubing Set No.	

B1 - Bin 1
 B2 - Bin 2
 B3 - Bin 3
 L1 - Load 1
 L2 - Load 2

Postop Orders:

- ① Propped up
 - ② Vigamox BID
 - ③ Presforte BID / 8 times/day
 - ④ Optixim BID (RB)
 - ⑤ Homicle BID 3ms/day (RB)
 - ⑥ Hypersol BID TDS (RB)
 - ⑦ T. Combiflam 100
- ⑧ T. Pan 40, 100 x 3ys
 JH

Dr. Agarwal's Eye Hospital
NO. 112, TTK Road, Alwarpet, Chennai-600 017
VITREO RETINAL SURGERY

NAME: SUMAZHAI. V AGE/SEX: 63. F REFNO:

SON/DAUGHTER: ANJALI. K. S DATE: 14/09/12 EYE:

ADDRESS: TEL: 9886422002

Proposed Treatment

The doctor has explained that I, (name of patient), have in my Eye and that is proposed.

Risks

These are the commoner risks. There may be other unusual risks that have not been listed here.

I understand there are risks associated with any anesthetic agent.

I may have side effects from any of the drugs used.

I understand the procedure has the following specific risks and limitations:

1. Failure to accomplish intent of surgery
2. Retinal detachments that may require additional surgery or may be inoperable
3. Depending upon the surgery, Silicone Oil or Gas may be required for tamponade.
4. In case of Silicone Oil or Gas injection, I have to maintain position depending upon the surgery
5. If Gas is injected, I have to restrict air travel until gas is absorbed.
6. If Silicone oil is injected then resurgery will be required to remove the oil.
7. It may take up to 18 months before the final outcome of the surgery is known.
8. In a few cases, the underlying condition cannot be treated and blindness cannot be prevented.
9. It may not be possible to predict before the operation which cases will do well.
10. There is a chance I may develop further retina detachments in future in the same eye or in the opposite eye.
11. In some cases, more than one operation may be required
12. Though rare I may develop complications like vitreous hemorrhage, infection, elevated eye pressure (glaucoma), poorly healing or non-healing corneal defects, corneal clouding and scarring, cataract, which might require eventual or immediate removal of lens, double vision, eyelid droop, and loss of circulation to vital tissues in the eye, resulting in decrease or loss of vision

There is an extremely small risk (1:17000 cases) that the opposite eye to the one having surgery may become inflamed, especially if complications occur after the operation. This is called sympathetic ophthalmia. Although this can be treated, in some cases, eyesight may be lost.

I understand some of the above risks are more likely if I smoke, am overweight, diabetic, have high blood pressure or have had previous heart disease.

Individual Risks

I understand the following are possible significant risks and complications specific to my individual circumstances, that I have considered in deciding to have this operation:

Declaration by Patient

I acknowledge doctors from the ophthalmic team have informed me about the procedure, alternative treatments and answered my specific queries and concerns about this matter.

I acknowledge that I have discussed with the surgical team any significant risks and complications specific to my individual circumstances that I have considered in deciding to have this operation.

I agree to any other additional procedures considered necessary in the judgment of my surgeon during this operation.

I agree to the disposal by the hospital authorities of any tissues that may be removed during the procedure. I understand that some tissues or samples may be kept as part of my hospital records.

I have received no guarantee the operation will be successful I have received a copy of this form to take home with me.

If a needle stick/sharps injury occurs to staff during any operation I give my permission for blood to be taken and tested for HIV and other blood borne disorders.

I understand I will be advised and counselled as soon as practicable after the operation if this has been necessary.

Signature / Thumb Impression of Patient/ Parent / Guardian: 

Name: Relationship: Date:

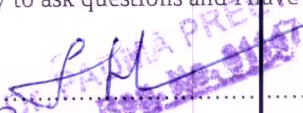
Address:

Phone (Off): (Res): (Mob):

Declaration by Doctor

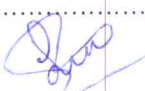
I declare that I have explained the nature and consequences of the procedure to be performed, and discussed the risks that particularly concern the patient.

I have given the patient an opportunity to ask questions and I have answered these.

Doctor's signature: 

Doctor's name: DR. PADMA

Date: Time:

Witness 1 

Witness 2

Signature: Signature:

Name: Name:

Address: Address:

Tel: Tel:

Dr Agarwals

Eye Hospital

CONSENT FOR LOCAL ANESTHESIA SERVICES (AEH/F/OT/4)

Name : MRS. Sumalhi. V

Age / Sex : 63/F

M.R. No. :

Son / Daughter :

Date :

CMH-239329-23

Eye : Right ☒

Left ☐

Vit + MPT + ERMP + Cat + AFL

I understand that anaesthesia services are needed so that my Doctor can perform the operation, procedure or treatment.

It has been explained to me / guardian that all forms of anaesthesia involve some risks and no guarantee or promises can be made concerning the results of my operation, procedure or treatment. Although rare, severe unexpected complications can occur with each type of anaesthesia, infection, bleeding, drug reactions, blood clots, loss of sensation and loss of vision, paralysis, stroke, brain damage, heart attack or death. I understand that these risks apply to all forms of anaesthesia and that additional or specific risks have been identified below as they may apply to a specific type of anaesthesia. I understand the anaesthetic technique to be used is determined by many factors including my physical condition, the type of procedure my Doctor is to do, his or her preference, as well as my own desire.

Major / Minor Nerve Block	Expected Results	Temporary decreased or loss of pain sensation and / or movement to the part of the body. May be combined with IV sedation, which will likely cause partial or total amnesia.
	Technique	Drug injected near nerves providing loss of pain sensation to the area of the operation.
	Risks (include but not limited to)	Infection, convulsions, weakness, persistent numbness, residual pain requiring additional anaesthesia, injury to blood vessels, incomplete or failed block, perforation of globe.

I hereby consent to receive services and authorized that it be administered by an Anaesthesiologist from Dr. Agarwal's Eye Hospital, all of whom are credentialed to provide anaesthesia services at this health facility. I also consent to an alternative type of Anaesthesia, if necessary, as deemed appropriate by the anaesthesiologist.

Patient / Guardian Signature Date :/...../..... Time :

Witness's Signature & Name : Date :/...../..... Time :

Surgeon's Signature & Name : Date : 16/9/23 Time : 11:00 AM



CRITERIA TO TRANSFER THE PATIENT FROM RECOVERY AREA (AEH/F/OT/8)

Patient's Name :	MRS. Sumathy .V		
M.R. Number :	Cm14-239329-23		
Surgery :	RE vit + mp + Eamp + Cat + AFL	Eye :	LEFT ☒ RIGHT ☒
Type of Anaesthesia :			

PATIENT RECOVERED FROM ANAESTHESIA :

YES ☒ NO ☐

VITAL SIGNS :

TEMP :	B.P : 130/80	PULSE : 78/min	RESP. RATE : 14/min
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NAUSEA :

PRESENT ☐

ABSENT ☒

PAIN STATUS

The pain assessment tool is intended to help patient care providers assess pain according to individual patient needs. Explain and use 0 - 10 Scale for patient self assessment. Use the faces or behavioral observations to interpret expressed pain when patient cannot communicate his/her pain intensity.

	0	1	2	3	4	5	6	7	8	9	10
Verbal Descriptor Scale	NO PAIN		MILD PAIN		MODERATE PAIN		MODERATE PAIN		SEVERE PAIN		WORST PAIN POSSIBLE
WONG-BAKER FACIAL GRIMACE SCALE											
	Alert Smiling		No humor serious flat		Furrowed brow pursed lips breath holding		Wrinkled nose raised upper lips rapid breathing		Slow blink open mouth		Eyes closed moaning, crying
ACTIVITY TOLERANCE SCALE	NO PAIN		CAN BE IGNORED		INTERFERES WITH TASKS		INTERFERES WITH CONCENTRATION		INTERFERES WITH BASIC NEEDS		BEDREST REQUIRED

AVAILABILITY OF ESCORT : YES ☒ NO ☐

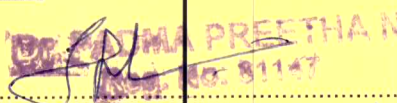
COMMUNICATED TO PATIENT OR ESCORT ON POST OPERATIVE CARE : YES ☒ NO ☐

NEXT FOLLOWUP ASSESSMENT DATE :

PLACE :

POST OPERATIVE INSTRUCTIONS :

FIT FOR DISCHARGE : YES ☒ NO ☐

SURGEON'S SIGNATURE : 

SURGEON'S NAME : DR. PADMA DATE : 16/9/23 TIME : 1.15 PM

Dr Agarwals

Eye Hospital

TOPICAL / IV SEDATION / LOCAL ANESTHESIA ASSESSMENT

PRE ANESTHESIA ASSESSMENT (AEH/F/OT/7)

Name : MRS. Sumathi . V .

Age : 63

Sex : F

RE VIT + MP + ERMP + CAT + AFL

M.R.No. :

Date :

Time :

Eye : Right ☒ Left ☐

History & Patient current Medication :

Temperature :

P.R. : 69 bpm

Respiration Rate :

B.P. : 158/89 mmHg

Drug Allergy :

SpO2 : 98 %

Pre-operative Medication :

Type of Anesthesia : TA / LA / IV Sedation

IMMEDIATE PRE OPERATIVE RE-EVALUATION

Temperature :

P.R. : 70/min

Respiratory Rate : 14/min

B.P. : 135/80 mmHg

Heart Rate :

SPO2 : 100 %



Dr Agarwals

Eye Hospital

INTRA OPERATIVE ASSESSMENT

Temperature :

P.R. : 76/min

Respiratory Rate : 14/min

B.P. :

130/80

Heart Rate :

Cardiac Rhythm :

normal

Airway Security & Patency :

ETCO 2 :

Level of Sedation :

Time :	10	15	20	25	30	35	40	45
Level of Sedation :								

Medications :

TIME	0	5	10	15	20	25	30	35	40	45	50	55	60					
B.P.	130/80																	
P.R.	76/min																	
SPO2	100%																	

POST ANESTHESIA ASSESSMENT

P.R. : 76/min

Respiratory Rate : 14/min

B.P. : 130/80 mmHg

SPO2 : 100%

Drug Allergy :

Instructions :

Dr. Agarwals
No. 104
Doctor Signature

Time : 10:40 PM

SURGICAL SAFETY CHECKLIST (AEH/F/OT/9)

BEFORE INDUCTION OF ANAESTHESIA		BEFORE INCISION		BEFORE PATIENT LEAVES OPERATION THEATRE	
SIGN IN		TIME OUT		SIGN OUT	
☐ PATIENT HAS CONFIRMED IDENTITY ● NAME : ● MRD NO.: ● SITE : ● PROCEDURE: ● CONSENT/LA/GA:		☐ CONFIRM ALL TEAM MEMBERS HAVE INTRODUCED THEMSELVES, THEIR ROLE AND NAME ☐ SURGEON, ANAESTHETIST, PROFESSIONALS AND NURSE VERBALLY CONFIRM ● PATIENT ● SITE ● PROCEDURE		☐ NURSE VERBALLY CONFIRMS WITH THE TEAM ☐ THE NAME / THE PROCEDURE RECORDED ☐ THE INSTRUMENTS, SPONGE AND NEEDLE COUNTS ARE CORRECT OR NOT APPLICABLE ☐ HOW THE SPECIMEN IS LABELED INCLUDING PATIENT NAME OR NOT APPLICABLE.	
☐ SITE MARKED / NOT APPLICABLE OD / OS / OU		☐ ANTICIPATED CRITICAL EVENTS ☐ SURGEON REVIEWS : WHAT ARE THE CRITICAL STEPS OR UNEXPECTED STEPS, OPERATIVE DURATION. ☐ ANAESTHESIA TEAM REVIEWS : ARE THERE ANY PATIENT SPECIFIC CONCERNS ?		☐ WHETHER ANY EQUIPMENT PROBLEM TO BE ADDRESSED ? ☐ YES ☐ NO	
		☐ NURSING TEAM REVIEWS : HAS STERILITY INCLUDING INDICATOR RESULTS BEEN CONFIRMED? REVIEWS : WHAT ARE THE CRITICAL OR UNEXPECTED STEPS ? ● OPERATIVE DURATION.		☐ IF YES, SPECIFY :	
☐ ANAESTHESIA FITNESS AVAILABILITY ☐ CARDIAC MONITOR ON PATIENT AND FUNCTIONING		HAS ANTIBIOTICS PROPHYLAXIS BEEN GIVEN WITH IN THE LAST 60 MINUTES ? ☐ YES ☐ NOT APPLICABLE			
DOES PATIENT HAVE A KNOWN ALLERGY ? ☐ NO ☐ IF YES, SPECIFY		IS ESSENTIAL DRAWING DISPLAYED ? ☐ YES ☐ NOT APPLICABLE		☐ SURGEON, ANAESTHESIA PROFESSIONAL & NURSE REVIEW THE KEY CONCERNS FOR RECOVERY AND MANAGEMENT OF THIS PATIENT	
SIGN 		SIGN 		SIGN 	
TIME		TIME		TIME	

SURGICAL SAFETY CHECKLIST (AEH/F/OT/9)

BEFORE INDUCTION OF ANAESTHESIA		BEFORE INCISION		BEFORE PATIENT LEAVES OPERATION THEATRE	
SIGN IN		TIME OUT		SIGN OUT	
☐ PATIENT HAS CONFIRMED IDENTITY ● NAME : ● MRD NO.: ● SITE : ● PROCEDURE: ● CONSENT/LA/GA:		☐ CONFIRM ALL TEAM MEMBERS HAVE INTRODUCED THEMSELVES, THEIR ROLE AND NAME ☐ SURGEON, ANAESTHETIST, PROFESSIONALS AND NURSE VERBALLY CONFIRM ● PATIENT ● SITE ● PROCEDURE		☐ NURSE VERBALLY CONFIRMS WITH THE TEAM ☐ THE NAME OF THE PROCEDURE RECORDED ☐ THE INSTRUMENTS, SPONGE AND NEEDLE COUNTS ARE CORRECT OR NOT APPLICABLE ☐ HOW THE SPECIMEN IS LABELED INCLUDING PATIENT NAME OR NOT APPLICABLE.	
☐ SITE MARKED / NOT APPLICABLE OD / OS / OU		☐ ANTICIPATED CRITICAL EVENTS ☐ SURGEON REVIEWS : WHAT ARE THE CRITICAL STEPS OR UNEXPECTED STEPS, OPERATIVE DURATION. ☐ ANAESTHESIA TEAM REVIEWS : ARE THERE ANY PATIENT SPECIFIC CONCERNS ?		☐ WHETHER ANY EQUIPMENT PROBLEM TO BE ADDRESSED ? ☐ YES ☐ NO	
		☐ NURSING TEAM REVIEWS : HAS STERILITY INCLUDING INDICATOR RESULTS BEEN CONFIRMED? REVIEWS : WHAT ARE THE CRITICAL OR UNEXPECTED STEPS ? ● OPERATIVE DURATION.		☐ IF YES, SPECIFY :	
☐ ANAESTHESIA FITNESS AVAILABILITY ☐ CARDIAC MONITOR ON PATIENT AND FUNCTIONING		HAS ANTIEMETICS PROPHYLAXIS BEEN GIVEN WITH IN THE LAST 60 MINUTES ? ☐ YES ☐ NOT APPLICABLE			
DOES PATIENT HAVE A KNOWN ALLERGY ? ☐ NO ☐ IF YES, SPECIFY		IS ESSENTIAL DRAWING DISPLAYED ? ☐ YES ☐ NOT APPLICABLE		☐ SURGEON, ANAESTHESIA PROFESSIONAL & NURSE REVIEW THE KEY CONCERNS FOR RECOVERY AND MANAGEMENT OF THIS PATIENT	
SIGN TIME		SIGN TIME		SIGN TIME	

Dr Agarwals

Eye Hospital

CONSENT FOR GUARDED VISUAL PROGNOSIS (AEH/F/OT/10)

Name : MRS. V-Sumathu

M.R. No. : CMH-239329-23 Age / Sex : 63 / F

Guarded Visual Prognosis must be explained to the Patients in the following conditions : RE vit + mp + ERmp + cat + AFL

1. If the fundus view is very hazy or no view :
2. Pallor of the disc :
3. Glaucoma :
4. Drusen :
5. NPDR (Non Proliferative Diabetic Retinopathy) / PDR :
6. BRVO (Branch Retinal Vein Occlusion) :
7. CRVO (Central Retinal Vein Occlusion) :
8. Retinitis pigmentosa :
9. Macular Hole :
10. High Myopia :
11. Any macular lesion :
12. RPE defects in the macula :
13. History of Retinal Detachment in the other eye surgery :
14. History of post operative CME in the other eye :
15. Bad Cornea :
16. Squint :
17. Defective vision in the eye to be operated from childhood :
18. Pterygium covering more than half of cornea :

"I HEREBY CONSENT THAT THIS IS A RESCUE OPERATION. THE VISUAL RESULT CANNOT BE FULLY GUARANTEED BECAUSE OF SUSPECTED / EXISTING NERVE / CORNEAL DAMAGE (DEPENDING ON THE CASE)"

(Consent form should be written by the patient in their own hand writing and signed by them as above.)

I HEREBY CONSENT THAT THIS IS A RESCUE OPERATION. THE VISUAL RESULT CANNOT BE FULLY GUARANTEED BECAUSE OF SUSPECTED / EXISTING NERVE / CORNEAL DAMAGE

Patient / Guardian Signature : [Signature] Date : 16/9/23 Time : 11-30 PM

Witness's Signature & Name : [Signature] Date : 16/9/23 Time : 11-30 PM

Surgeon's Signature & Name : [Signature] Date : 16/9/23 Time : 11-30 PM

1, 2, 3

1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12

1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12

1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12

1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12

1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12

1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12

MRS. V. Sumathi

RE VIT + mp + ERMP - Cat + AFL

63 / F
Dr Agarwals
Eye Hospital

CMH - 239329-23

**டாக்டர் அகர்வால் கண் மருத்துவமனை
HIV சோதனைக்கான ஒப்புதல் படிவம்**

**DR. AGARWAL'S EYE HOSPITAL
CONSENT FOR HIV TEST**

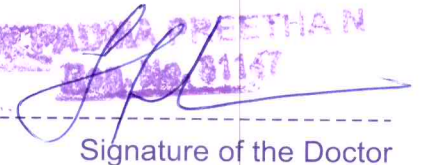
மருத்துவர்கள் / ஆலோசகர்களின் ஆலோசனை படி, நான்
இந்த HIV பரிசோதனை செய்து கொள்ள சம்மதம்
தெரிவிக்கிறேன்.

I hereby express my willingness to undergo HIV Test.
I have been counseled by Doctors / Counselors
to undergo these Investigations.



நோயாளி (அல்லது) பாதுகாவலர் கையொப்பம்
Signature of the Patient (or) Guardian

DR. PADMA PRUTHI N
239329-23



Signature of the Doctor

DrA

Regd. Office : 1st & 3rd Floor, Buhari Towers, No.4, Moore Road, off Grems Road, Near Asan Memorial School,
Chennai - 600 006. Contact : 78248 19990
Email : info@dragarwal.com Website : www.dragarwal.com

1/1/20

1/1/20

1/1/20

1/1/20

1/1/20

CATARACT SURGERY WITH/ WITHOUT IMPLANTATION OF INTRAOCULAR LENS

NAME: SUMATHI - V AGE/SEX: 63 REF NO: CMH-239329/23
 SON/DAUGHTER: ANIL - K S DATE: 14/09/23
 ADDRESS: 3/218, KAUPPADA VILA, Pachavathan Kodu, THARAMBUR TEL: 9886422002

INTRODUCTION:

A cataract is opacity of the lens. Cataract operation is indicated only when you cannot function adequately due to poor sight produced by the cataract. Maturity of cataract is no longer a criterion for surgery. The natural lens with your eye with a slight cataract, although not perfect, has distinct advantages over an artificial lens. In giving permission for cataract extraction with/ without implantation of an intraocular lens in my eye, I declare that I understand the following information

1. Alternative treatments:

There are Two methods of restoring vision

a) Spectacles b) Contact lens

Spectacles increase image size by 30%. They cannot be used if there is cataract in only one eye (the other is normal) because they may cause double vision. A contact lens increases image size by 8%. However, it is difficult to handle and may not be tolerated by everyone. Intraocular lens does not increase image size. It is surgically placed inside the eye permanently.

2. An intraocular lens is implanted by surgery (not by laser). The implanted lens will be left in the eye permanently. At the time of surgery the doctor may decide not to implant an intraocular lens in the eye, if for any reason he feels that the lens implantation is not indicated or may prove deleterious to the well being of the eye, even though permission may have been given to do so.
3. Though the intraocular power is calculated by utilizing a computerized Biometer (A- Scan), a small correction in the spectacles is to be considered invertible postoperatively and this may be more in specific cases. An astigmatism (number with axis) which may reduce with time is to be taken as inevitable and normal. Therefore, a small power is to be expected in the spectacles for distance and near for clear vision after the operation. In any case the aim of cataract surgery is to remove the cloudy lens from the eye and replace it with a plastic lens and not to get rid the patient of his spectacles..
4. The caliber of vision obtained after a successful cataract surgery/ lens implantation depends upon the retina behind. In an advanced cataract even with the most sophisticated instruments (ultrasound scan etc), it is not possible to be certain that the retina inside is normal. Removal of cataract is like opening a door to a room. If the retina is normal, you will see well, but it is not possible in a majority of advanced cataract cases to ascertain the visual status of retina before operation.
5. With modern instrumentation and micro surgical techniques, the rate of complications in cataract surgery with/ without intraocular lens implantation is very low. Complications can usually be managed by medical and/ or surgical treatment. The chances of total loss of vision are less than 0.5%. However, the following complications can occur and are mentioned in standard text books of cataract and lens implantation surgery.
 - a) It is possible that vision may drop after surgery due to thickening/ opacification of the posterior capsule. This not a complication but a sequelae to extra capsular cataract extraction. The condition is treated with the "Yag Laser"
 - b) Complication may include haemorrhage (bleeding), posterior capsule rupture, nucleus drop, vitreous loss, wound leakage, uveitis, corneal decompensation, glaucoma, cystoid macular oedema or retinal detachment. In addition lens implantation may be complicated by severe reaction to the lens (toxic lens syndrome) or dislocation of the lens. The implanted lens may have to be repositioned or removed surgically if it is likely to damage the eye. Though every effort is made to minimize the chances of infection, it cannot be eliminated altogether. Loss of vision is a risk common to any intraocular surgery.
 - c) Although you may have opted for phacoemulsification surgery and the same may have been planned by your surgeon after pre operative examination, if during surgery phacoemulsification is found to be unsafe or not feasible. Your surgeon will have the liberty to perform surgery by the conventional technique in the interest of patient's safety.
 - d) Complications of surgery in general: As the procedure is generally done under local anaesthesia the risk to life is less than 0.5%. Risk is greater in patient with diabetes, hypertension, cardiac ailments and other systematic disorders & when surgery is performed under general anaesthesia. There is a possibility of drug reaction, brain damage or risk to life.

Since it is impossible to state every complication that may occur as a result of surgery, the list of complications in this form is not exhaustive.

CONSENT FOR OPERATION:

1. I hereby authorize Dr. and those whom he may designate as associates or assistants to perform cataract operation with / without intraocular lens/ as a secondary procedure on my left/ right eye.
It has been explained to me that during the course of operation/ procedure, unforeseen conditions may be revealed or encountered which necessitate surgical or other procedures in addition to or different from those contemplated. I, therefore, further request and authorize the above mentioned physician/ surgeon or his designates to perform such additional surgical or other procedures as he or they deem necessary or desirable.
2. The nature and purpose of the operation, the necessity thereof, the possible alternative methods of treatment of my condition have been fully explained to me and I understand the same.
3. I am fully aware that the surgery is being performed in good faith and that no guarantee or assurance has been given as to the result that may be obtained.
4. I consent to the administration of such drugs or infusions deemed necessary in the judgment of the medical staff.
5. Any tissues or parts surgically removed may be disposed off by the institution in accordance with customary practice.

I (the patient) hereby give my consent for the operation of left eye/ right eye with the full knowledge of possible complications.
I certify that i have read this informed consent. it has been read over to me and explained to me in my understandable language and all blanks or statements requiring insertion or completion were filled in and inapplicable paragraphs stricken off before i signed. the doctor has answered all my questions to my satisfaction.

Signature/ Thumb impression of Patient/ Parent/ Guardian:

Name: Relationship: Date:

Address:

Contact numbers :

DECLARATION BY DOCTOR :

I declare that I have explained the nature and consequences of the procedure to be performed, and discussed the risks that particularly concern the patient. I have given the patient an opportunity to ask questions and I have answered them to the satisfaction of the Patient.

Doctor's Signature:

Doctor's Name:

Date:

Time:

Witness 1

Signature:

Name:

Address:

Tel:

Witness 2

Signature:

Name:

Address:

Tel: